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PREPARED FOR THE

FEDERAL MINISTRY OF HEALTH AND SOCIAL WELFARE

DoctaRx Abuja Pilot Program

AI-Enabled Digital Health Coordination Platform for a Controlled FCT Pilot

PRESENTATION FLOW

1. Executive Overview
2. Pilot Model & Workflow
3. Implementation, Governance & Budget
4. Decision Pathway

8 July 2026

DoctaRx | Abuja, FCT, Nigeria

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WHY WE ARE HERE

Executive Overview

A focused presentation on the DoctaRx Abuja/FCT pilot opportunity and implementation model.

Meeting Purpose

- Present the DoctaRx proposal in operational, clinical, technical and governance terms
- Demonstrate the platform workflow and pilot design
- Align on next steps for pilot planning under Ministry guidance

Scope of Presentation

- PHC-supported intake and triage
- Provider review and video consultation
- Prescription, referral, follow-up, dashboards and reporting readiness

Expected Meeting Outcomes

- Shared understanding of pilot scope and value
- Agreement on technical working session and focal persons
- Direction on facilities, indicators and onboarding priorities

Pilot At a Glance

Sites	2–3 PHCs + 1 referral hospital pathway
Duration	12 months
Users	PHC teams, clinicians, pharmacy partners, Ministry focal persons
Outputs	referral visibility, teleconsultation support, reporting readiness, dashboard insight
Primary ask	confirm pilot planning session and implementation pathway

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THE OPPORTUNITY

Current Health System Gaps

The pilot targets practical coordination frictions that can be measured and improved.

1. Fragmented access

- Patients often do not know where to start
- PHCS lack fast escalation options
- Care pathways are not consistently visible

2. Weak referral closure & follow-up

- Referrals are hard to track to endpoint
- Missed follow-ups are often invisible
- Facilities struggle to confirm continuity of care

3. Limited remote clinical support

- Specialist input is not always available in time
- Video-enabled consultation workflows are inconsistent
- Clinical review and documentation are not streamlined

4. Delayed administrative insight

- Leaders rely on late or incomplete reports
- Facility workload and service patterns are hard to see
- Reporting formats vary across teams

Why this matters

- Patients lose time and clarity
- Facilities duplicate effort
- Clinicians lack continuity signals
- Leadership sees problems too late

Pilot objective: strengthen coordination without disrupting existing care structures.

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OPERATING MODEL

DoctaRx Operating Workflow

A controlled pilot supports a practical care journey with an implementation-ready workflow.

1 Request capture

Patient request is captured at PHC or portal

2 PHC intake & triage

Basic context and routing are confirmed

3 Doctor review & teleconsultation

Clinician reviews case and conducts video consultation where appropriate

4 Care action

Notes, prescription, referral, or follow-up are documented

5 Referral & follow-up

Hospital or specialist pathway receives a structured summary and follow-up status is tracked

6 Dashboard & reporting

Leadership sees aggregate indicators and monthly reporting outputs

What the pilot enables

- Structured patient journey from intake to follow-up
- Role-based workflow for PHC teams, clinicians, and partners
- Aggregate Ministry-level visibility and monthly reporting
- Integrated support for referral, teleconsultation, prescription, and follow-up

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PLATFORM CAPABILITIES

Core Platform Modules

The pilot uses a focused set of modules that strengthen coordination, visibility, and reporting readiness.

1. Referral & Care Coordination

- End-to-end referral workflow
- Follow-up tracking and continuity of care

2. Teleconsultation & Virtual Triage

- Secure video consultation support
- Remote specialist input where appropriate

3. Prescription & Pharmacy Coordination

- Prescription visibility and status tracking
- Safer dispensing coordination

4. Public Health Dashboards & Analytics

- Aggregate service and utilization insight
- Response time and workload visibility

5. NHMIS / DHIS2 Reporting Support

- Structured data mapping and export readiness
- Approval-led reporting workflow

6. Nursing Education & Research Collaboration

- Supports academic collaboration and workforce development
- Useful for future university and training partnerships

Cross-cutting capabilities | role-based access | audit trail | support desk | training | configurable workflows | monthly reporting pack

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PILOT DESIGN

Pilot Scope, Benefits & KPIs

A controlled Abuja/FCT pilot gives the Ministry evidence for a confident implementation pathway.

Controlled scope

- 2–3 PHCs
- 1 referral hospital pathway
- Selected clinicians and pharmacy partners
- Support desk and monthly review cycle

Key beneficiaries

- Patients
- PHC teams
- Clinicians
- Pharmacy partners
- Ministry focal persons

Expected benefits

- Better referral completion and continuity of care
- Faster follow-up visibility
- Improved access to teleconsultation support
- Better dashboard visibility for leadership
- Foundation for broader digital-health scale-up

KPI	Definition	Target
Referral completion rate	% of referrals completed at destination facility	>=70%
Follow-up visibility	% of referred patients with known next-step status	>=70%
Teleconsultation utilization	% of eligible consultations conducted via telehealth	>=50%
Reporting completeness	% of pilot events represented in dashboard/reporting	>=90%
Average response time	Average time from request capture to first clinical response	<=24 hours
Staff adoption	% of targeted pilot staff onboarded and active	>=80%

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GOVERNANCE

Governance, Privacy & Security

The pilot is supported by clear ownership, approved access controls, and Ministry-guided reporting rules.

1 OPERATING GOVERNANCE

Ministry / DHS
approval, oversight, focal persons, review cadence

Facilities
PHC intake, referral participation, frontline feedback

Clinicians
review, consultation, documentation, prescriptions, referral decisions

Pharmacy partners
prescription visibility and fulfillment-status workflow

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configuration, training, support, dashboard, reporting pack

Data governance lead
consent model, role access, audit review, report approvals

2 PRIVACY & SECURITY GUARDRAILS

- NDPA-aligned privacy review
- Role-based access and least-privilege permissions
- Separation of test and live environments
- Audit logs, backup, and downtime procedure
- Reporting elements agreed before scale-up or integration

Implementation is supported by Ministry guidance, agreed governance, and strong data protection standards.

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WORK PLAN

Implementation Timeline & Work Plan

A phased 12-month rollout supports alignment, controlled launch, evaluation, and scale planning.

MONTH	1	2	3	4	5	6	7	8	9	10	11	12
Phase 1 Alignment — Months 1–2	■	■	• Stakeholder workshop • Site selection • Focal persons • Indicator mapping									
Phase 2 Configuration — Months 2–4		■	■	■	• Workflow tuning • Sandbox setup • User roles • Training preparation							
Phase 3 Controlled Launch — Months 5–8					■	■	■	■	• Go-live at selected facilities • Support desk activation • Onboarding • Bi-weekly reviews			
Phase 4 Evaluation — Months 9–10									■	■	• KPI review • Issue resolution • Workflow improvement • Assessment report	
Phase 5 Scale Planning — Months 11–12											■	■ • Cost model • Scale recommendation • Roadmap for expansion

Core deliverables

- Agreed pilot governance note
- Operational sandbox and configured workflows
- Staff training and onboarding pack
- Monthly progress briefs
- Pilot assessment report
- Expansion roadmap under Ministry guidance

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BUDGET

Indicative 12-Month Pilot Budget

Budget framework aligned to pilot delivery requirements across technology, operations, training, and reporting.

Category	Amount (₦ millions)
Product configuration & engineering	22.0
Pilot operations & local coordination	18.0
Devices, connectivity & power resilience	15.5
Clinical support & supervision	12.0
Training & onboarding	9.5
Reporting, monitoring & evaluation	8.4
Transport, meetings & field logistics	7.5
Contingency / inflation reserve	7.5
Governance, compliance & data protection	6.0
Total indicative budget	**106.4**

Budget logic

- Configuration and workflow tuning for selected sites
- Local operations and support desk for pilot execution
- Devices, data, and backup power resilience
- Clinical supervision, onboarding, and reporting
- Compliance, privacy, and contingency for inflation and logistics

Budget allocation (₦ millions)

Product & engineering	22.0	(20.7%)
Pilot operations	18.0	(16.9%)
Devices & power resilience	15.5	(14.6%)
Clinical support	12.0	(11.3%)
Training & onboarding	9.5	(8.9%)
Reporting & M&E	8.4	(7.9%)
Transport & logistics	7.5	(7.1%)
Contingency reserve	7.5	(7.1%)
Governance & compliance	6.0	(5.6%)

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DECISION PATHWAY

Pilot Readiness, Sustainability & Next Steps

The presentation outlines a clear, practical pathway for launching a successful Abuja/FCT pilot.

Implementation success factors

- Phased rollout with focused facility onboarding
- Reliable connectivity planning and support desk
- Clear governance and reporting approvals
- Stage-based budget management

Sustainability logic

- Local support team and training-of-trainers model
- Monthly reporting cadence and facility focal persons
- Clear cost logic per facility and user cluster
- Scale decision based on evidence of adoption and reporting readiness

Requested decisions

- Confirm Ministry and pilot focal persons
- Select 2–3 PHCs and one referral pathway
- Approve pilot indicators and reporting pack
- Schedule technical working session and onboarding plan
- Authorize controlled pilot launch once governance conditions are met